SURYA TEJAS YOGALAYAM

Patient Case History & Yoga Therapy Management System

For STY Vijayawada and STY Hyderabad | Draft for clinical and technical review

1. Purpose of this document

This document specifies exactly what the developer should build in the STY online patient-record system: the screens, subsections, fields, field types, workflow, validations, permissions and reports.

2. Product objective

Build a secure, mobile-friendly online system used by both STY Vijayawada and STY Hyderabad. Each patient must have one longitudinal record, while assessments, investigations, therapy plans, individual sessions, reviews and follow-ups are saved as separate dated records linked to that patient.

- Capture patient-reported history separately from professional observations and documented medical diagnoses.
- Record what was planned and what was actually delivered in each therapy session.
- Store investigation metadata and secure file references for X-rays, MRI reports and other documents.
- Track outcomes from baseline through review and discharge.
- Allow access by role and centre; do not give every staff member unrestricted access to medical records.
- Generate useful case summaries and operational reports without retyping saved information.

3. Main menu / application modules

Menu item	Purpose	Records created
Dashboard	Centre-wise overview, tasks due, appointments and active cases.	Dashboard views; no duplicate patient data
Patients	Register, search, view and update a patient profile.	Patient profile
Consent & Privacy	Capture and maintain consent by purpose.	Consent records and versions
Case History	Chief complaint, symptoms, medical history and lifestyle.	Dated case-history entries
Assessment & Safety	Physical assessment, region-specific tests and safety screen.	Assessment and screening records
Investigations	Record reports and attach files securely.	Investigation records and attachments
Therapy Plan	Create/revise an individualized plan and home programme.	Versioned therapy plans
Session Log	Document each actual visit and intervention.	One record per session
Progress Review	Compare baseline and current outcomes.	Review records
Discharge & Follow-up	Close, transfer or follow up the case.	Discharge and follow-up records
Technique Library	Maintain approved practices, props, instructions and precautions.	Reusable library items
Staff & Centres	Manage staff, qualifications, roles and centre assignment.	Staff and centre records
Reports & Audit	Generate summaries and track record	Reports and audit events

	access/changes.	

4. Detailed screen and field requirements

4.1 Patient registration

Subsection	Field to display/capture	Field type / rule
Identity	Patient ID	System-generated unique ID; e.g., STY-VJA-2026-0001. Never reuse.
Identity	Full name; preferred name	Text; full name required.
Identity	Date of birth; age	Date of birth preferred; age calculated automatically where possible.
Identity	Gender	Optional/select only if needed for care.
Contact	Mobile; email; address/locality; preferred language	Text; collect minimum necessary information.
Emergency contact	Name; relationship; phone	Text; mark optional unless clinical lead requires it.
Centre & allocation	Registered centre; current centre; assigned therapist; referred by	Dropdown/search; preserve centre-transfer history.
Lifestyle context	Occupation; work pattern; sitting/standing duration; physical activity	Structured choices plus notes.
Administration	Registration date; status; created by; last updated by	System-generated audit fields; status dropdown.
Duplicate check	Possible matching patient by phone/name/date of birth	Warn user; authorized staff confirm merge rather than silently merging.

Patient status values: New; Assessment Pending; Active; On Hold; Referred; Discharged; Closed. Allow authorized staff to change status with a recorded reason.

4.2 Consent and privacy

Consent item	Required information	Behaviour
Assessment/documentation	Consent status, date, method, person obtaining consent	Required before assessment records are created, subject to approved clinical workflow.
Yoga practice	Consent status and date	Record separately from general privacy consent.
Physical adjustments	Consent for hands-on assistance where applicable	Must not be bundled with other permissions.
Reports/data sharing	Purpose, recipient/type of professional, status and date	Restrict sharing to authorized recipients.
Media	Photo/video permission and specific purpose	Optional and separate; refusal must not block therapy.
Communications	Permission for calls, SMS, email or WhatsApp reminders	Store channel-specific preference.
Consent record	Patient/guardian name, relationship if applicable, signature or documented method, version, staff member	Keep history of changes and withdrawal; do not overwrite old consent.
Privacy requests	Access/correction/withdrawal request, date, owner, action and closure date	Track resolution and audit it.

4.3 Medical case history

Subsection	Fields / prompts	Input design
Chief complaint	Patient's own words; primary body region; side; main concern; first or recurring episode	Text plus dropdowns.

Onset & course	Onset date; duration; sudden/gradual; injury/activity associated; progression; previous episodes	Date, choices and narrative.
Pain/symptoms	Current/best/worst pain (0–10); quality; location/radiation; frequency; time pattern; stiffness; associated symptoms	Numeric score with scale label, multi-select and notes.
Aggravating/relieving factors	Movement, posture, work, rest, sleep or other factors	Multi-select plus free text.
Function	Walking, bending, lifting, sitting, standing, stairs, work, sleep or other limitations	Activity checklist; severity and notes.
Medical conditions	Known diagnoses, date/clinician if known, past injuries, surgery, hospitalization and relevant conditions	Repeatable entries with dates.
Medication/allergy	Patient-reported medicines, dose/frequency if known, prescriber if known, allergies and reaction	Repeatable entries; clearly label patient-reported.
Other providers	Treating doctor/specialist, other therapy, current restrictions	Text and optional contact details.
Lifestyle	Sleep hours/quality, exercise/walking, occupation, posture, work hours, relevant diet/hydration factors	Structured options plus notes; optional when not relevant.
Sensitive history	Pregnancy/postpartum, menstrual/menopause or other relevant history	Ask only when relevant, explain purpose and restrict access.
Patient goals	Desired activities, measurable goals, expectations, preferred frequency and barriers to attendance/home practice	Text and goal records.

The system must identify whether a statement is patient-reported, taken from a medical document, or recorded as a professional finding. Do not automatically convert patient statements into confirmed diagnoses.

4.4 Safety screening and escalation

Screening prompt	Fields to capture	System response
New/severe/rapidly worsening symptoms	Yes/No/Unknown; description; screening date; reviewer	Flag for review; follow approved escalation procedure.
Recent significant trauma/fall	Answer, date and details	Prompt appropriate medical assessment where indicated.
New/progressive weakness or altered sensation	Answer, location, onset and notes	Flag for qualified clinical review.
New bladder/bowel control issue or saddle-area sensory change	Answer and time identified	High-priority warning; advise urgent medical evaluation according to approved protocol.
Fever/systemic symptoms/unexplained weight loss or concerning night pain	Answer and details	Flag for clinical review.
Chest pain, breathlessness, fainting or other acute symptoms	Answer and details	Emergency warning and urgent local care instructions where appropriate.
Decision and action	Pause/modify practice; referral advised; referred to; urgency; date/time; person responsible; outcome	No automatic diagnosis or clearance.

A qualified clinical lead must approve the final red-flag list and exact warning messages. A positive answer must not be automatically treated as a diagnosis. The system must not declare a patient safe to practise solely from a questionnaire.

4.5 Assessment and tests

Subsection	Fields / prompts	Rules
Assessment metadata	Date/time; assessor; qualification/role; referral question; patient consent status	System timestamp plus staff selection.
General observations	Posture; gait; balance; mobility; relevant	Structured choices plus narrative; record

	vitals if taken	units.
Movement/function	Region; movement; range; measurement and unit; side; pain/limitation; method	Repeatable measurement rows; allow unavailable/not tested with reason.
Strength/sensation	Finding, side, scale/method, notes	Only by staff qualified and authorized to perform the assessment.
SLR and special tests	Test name; indication; side; procedure/protocol; result; angle/unit if relevant; date; assessor; interpretation	Test library configurable; interpretation attributed to qualified professional.
Functional outcomes	Measure name/version; score; units; date; assessor	Use the same measure/version at follow-up where possible.
Clinical impression	Assessment summary; limitations; working impression; source/author	Separate documented diagnosis from therapist observations.
Safety decision	Proceed/modify/pause/refer; rationale; reviewer	Record decision, responsible professional and timestamp.

Region-specific form sections should appear conditionally:

- Cervical spine: neck movement, stiffness, pain pattern, shoulder symptoms and relevant neurological screening.
- Thoracic spine: upper-back mobility, posture, pain and movement limitations.
- Lumbar spine: back movement, pain distribution, lower-limb symptoms and relevant neurological screening.
- Pelvis/hip: hip movement, gait, pelvic mobility and relevant functional tests.
- Knee: pain, swelling, range of motion, alignment, gait and sit-to-stand function.
- Shoulder: movement, functional reach, pain and strength where appropriate.
- Ankle/foot: mobility, balance, gait and weight-bearing tolerance.
- General wellness: flexibility, balance, endurance and functional movement when relevant.

4.6 Investigations and attachments

Field	Requirement
Investigation type	X-ray, MRI, CT, ultrasound, laboratory test or other.
Body region/side	Structured field; allow multiple regions if applicable.
Investigation date	Date shown on the investigation/report.
Provider	Hospital/diagnostic centre and ordering/referring professional if known.
Report details	Report findings and impression copied accurately from the report; do not have the app invent interpretations.
Clinical review	Reviewer name/qualification, review date and attributed notes.
File	Secure file upload or protected document reference; permitted formats and file-size limits configured.
Comparison	Link to earlier investigation and note what is being compared.
Follow-up	Follow-up investigation requested, reason, due date and status.

The application should not automatically diagnose from X-ray/MRI images. Images and reports must be accessible only to authorized users.

4.7 Therapy plan

Subsection	Fields / prompts
Plan metadata	Plan ID; patient ID; date; version; prepared by/qualification; clinical reviewer if required; linked assessment.
Goals	Short-term and long-term goals; baseline; target; target review date; status.
Schedule	Sessions per week; proposed total/duration; session length; review date.
Practice item	Technique/asana name; Sanskrit/common name; category; body region; purpose; library reference.
Props/setup	Belt, brick, rope, loop band, tennis ball, chair, bolster, wall or other; starting position; setup; support/assistance.
Dose	Repetitions; hold duration; sets; rest interval; frequency; side.

Instructions	Breathing/teaching cues; approved modifications; home-practice instructions.
Safety	Precautions; contraindications; stop criteria; referral/co-management plan.
Approval/versioning	Patient agreement where applicable; author/reviewer; approved date; version; reason for change; previous version retained.

Create a searchable, administrator-controlled technique library. Each library item should have name, category, description, equipment, setup, dose fields, precautions, contraindications, approved status, author/reviewer and version. Do not auto-prescribe a standard routine solely from a diagnosis.

4.8 Session log — one record per visit

Session subsection	Fields to capture
Visit details	Session ID; patient ID; centre; date; start/end time; therapist; qualification; attendance status.
Before session	New symptoms/medical changes reported; pre-session pain score; safety check; proceed/modify/pause decision.
Actual intervention	Technique/asana; props; position/setup; side; repetitions/holds/sets; assistance; modification; time spent.
Patient response	Tolerance; comfort; reported symptoms; functional observations; reason for stopping or modifying.
After session	Post-session pain score if appropriate; adverse event; education; home practice assigned/reinforced; next plan.
Escalation	Referral/escalation reason, recipient, urgency, action and follow-up owner.
Record accountability	Recorded by; timestamp; completion status; amendment history.

Provide a 'copy previous plan as draft' feature, but the therapist must confirm each intervention actually delivered. Never silently copy previous session results into a new session.

4.9 Progress review

Field	Requirement
Review metadata	Review date; reviewer; sessions planned/attended/missed; home-practice adherence.
Comparison	Baseline and current pain score; range/function measurements; same outcome measure and score where available.
Goal status	Each goal marked achieved/partially achieved/not achieved/not assessed, with notes.
Patient feedback	Reported improvement/worsening, daily-function changes, concerns and satisfaction if collected.
Plan decision	Continue/modify/pause/refer; reason; new plan version; reviewer and date.
Next action	Next review date; pending medical review; follow-up owner.

Charts should display actual dated values and units. Do not infer medical recovery from a single pain score or attendance count.

4.10 Discharge and follow-up

Subsection	Fields
Closure	Discharge/transfer date; status; reason; total planned/completed sessions; closing staff member.
Outcome	Original goals; final assessment; final pain/function measures; goals achieved/partially achieved/not achieved.
Instructions	Home programme; precautions; warning signs explained; patient understanding confirmed.
Summary	Discharge summary generated from saved records; report provided and date.
Follow-up	Due date; contact method; contact outcome; symptoms

	reported; home adherence; next action.
Referral	Recommended provider/service, reason, date, urgency and whether follow-up was completed.
Case closure	Outstanding items; closure approval if required; record remains retrievable under retention policy.

5. Administration, centres and staff roles

5.1 Centre records

- Centre name/code: STY Vijayawada or STY Hyderabad; address, contact details, active/inactive status, local operating settings.
- Centre transfers must preserve one patient ID and record transfer date, reason, authorizing staff and receiving centre.

5.2 Staff roles and permissions

Role	Allowed access (minimum necessary)
System administrator	Accounts, roles, configuration, centre setup and technical audit; clinical access only if separately authorized.
Centre administrator	Registration, appointments, staff assignment and operational reports for assigned centre.
Yoga therapist	Assigned patient history relevant to care, assessments within scope, plan and session documentation.
Doctor/clinical reviewer	Relevant history, investigation review, clinical review, referrals and approvals within professional scope.
Reception/operations	Minimum necessary contact, appointment, attendance and payment information; no unrestricted clinical notes.
Read-only auditor/lead	Approved reports and records required for oversight; access logged.

Permissions must be configurable. Staff should only see the records needed for their work. Cross-centre access must be explicitly authorized.

6. Dashboard and reports

Report/dashboard	Required data
Centre overview	New registrations, active cases, patients on hold, referred and discharged cases by centre and date range.
Appointments	Today's appointments, cancellations, missed visits and next appointments.
Therapist workload	Assigned cases, sessions delivered and documentation pending; avoid using raw counts as a clinical quality score.
Follow-up tasks	Overdue reviews, upcoming follow-ups, pending reports and unresolved referrals.
Patient progress	Baseline/current measurements, dated trend charts and goal status for authorized users.
Case summary PDF	Demographics required for care, complaint, relevant history, assessments, investigations, plan, session summary and follow-up.
Discharge summary	Reason, baseline/final outcome, completed sessions, home plan, precautions and referral/follow-up.
Audit report	Who accessed/created/changed/exported records, when, and what type of action occurred.

7. Application behaviour and validation rules

- Use dropdowns and repeatable structured fields where possible; provide narrative boxes for details that cannot be captured in a dropdown.
- Show conditional subsections based on selected body region, test or intervention category.
- Use consistent units and labels. For pain, show the 0–10 scale explanation; for measurements, store unit and method.
- Allow 'Not assessed', 'Not applicable' and 'Unknown' where appropriate, with a reason for safety-critical fields.
- Validate date order, numerical ranges, required fields and duplicate IDs.
- Do not allow session completion without session date, therapist, actual interventions, response and safety check, unless an authorized exception is recorded.
- Preserve history: changes to a signed/closed record should be amendments with author, timestamp and reason, not silent overwrites.
- Support secure file uploads, file type/size validation, virus scanning where supported, backup and recovery.
- Support export only for authorized roles and log exports.
- Provide clear error messages and save-draft functionality to reduce data loss.

8. Security, privacy and governance

- Use secure authentication, role-based access, encrypted transmission and protected storage.
- Apply automatic session timeout, strong account security and a documented account deactivation process.
- Keep audit logs for access and changes to sensitive records.
- Use a documented retention/deletion schedule and secure backup/recovery process.
- Obtain and record appropriate consent; provide a process for consent withdrawal and patient data requests.
- Do not put patient names, phone numbers, medical details or report links in public/shared links or unsecured chat groups.
- Use synthetic/demo data during development and testing, not real patient records.
- Have STY's clinical lead and appropriate privacy/legal adviser review the final workflows and applicable Indian requirements before launch.

9. Recommended database entities

Build separate linked entities/tables rather than one very large patient form:

- Centres; Staff; Roles & Permissions
- Patients; Consent Records; Case Histories
- Assessments; Safety Screenings; Test Results
- Investigations; Secure Attachments
- Therapy Plans; Plan Versions; Technique Library
- Treatment Sessions; Session Intervention Items
- Progress Reviews; Outcome Measurements
- Discharge Records; Follow-up Tasks; Referrals
- Appointments; Notifications; Audit Logs

Relationship rule: one patient can have multiple case-history entries, assessments, investigations, therapy-plan versions, sessions, progress reviews and follow-ups. Every child record must reference the Patient ID and have its own record ID, date and author.

10. Required screens / wireframes

1. Login and role-aware dashboard.
2. Patient search and patient profile with tabs: Overview, History, Assessments, Investigations, Plans, Sessions, Progress, Discharge and Documents.
3. New patient registration and duplicate warning.

4. Consent capture and consent-history view.
5. Case-history form with conditional fields.
6. Safety-screen form and escalation panel.
7. Assessment form with region-specific subsections and test-result rows.
8. Investigation list, secure upload and report metadata form.
9. Therapy plan editor with technique-library picker and version history.
10. Session log with planned-versus-actual intervention comparison.
11. Progress review with baseline/current comparison.
12. Discharge/follow-up workflow.
13. Staff/centre/permission administration.
14. Report builder, PDF preview and audit view.

11. Acceptance criteria — how STY will check the build

- A patient can be registered once and seen at either centre under the same ID with authorized access.
- Staff can enter a complete history, safety screen, assessment and investigation reference without putting everything into one notes field.
- A therapist can create a plan and record actual session activities separately.
- The system retains previous plan versions and dated assessment/session records.
- Progress reports compare actual baseline and follow-up values with dates and units.
- Authorized staff can generate case and discharge summaries without retyping the data.
- Reception users cannot access unrestricted clinical notes or medical attachments.
- Every relevant create, edit, access and export action is logged.
- Consent changes and withdrawal are recorded and reflected in relevant workflows.
- The system works on common mobile and desktop screen sizes and saves drafts reliably.

12. Development deliverables requested

- A Software Requirements Specification (SRS) confirming each field and workflow with STY.
- Screen-by-screen wireframes/prototype for approval before coding.
- Database diagram and field dictionary, including data types and mandatory/optional rules.
- Role/permission matrix and privacy/security plan.
- Working application in a test environment with synthetic data.
- Test cases and results for validation, permissions, backups, reports and centre transfer.
- User guide and staff training.
- Deployment, maintenance, backup/recovery and support plan.
- Itemized development cost, hosting/storage cost, timeline, exclusions and future enhancement estimates.

13. Clinical review and scope note

This document defines software fields and workflows; it does not prescribe treatment or establish clinical protocols. Before launch, STY's designated clinical lead must approve the final safety questions, tests, terminology, consent wording, clinical decision workflow and report templates. Diagnosis, imaging interpretation, referral decisions and medical clearance must be attributed to appropriately qualified professionals. Collect only information necessary for care and protect it accordingly.

Patient registration

Subsection	Field to display/capture	Field type / rule
Identity	Patient ID	System-generated unique ID; e.g., STY-VJA-2026-0001. Never reuse.
Identity	Full name; preferred name	Text; full name required.
Identity	Date of birth; age	Date of birth preferred; age calculated automatically where possible.
Identity	Gender	Optional/select only if needed for care.
Contact	Mobile; email; address/locality; preferred language	Text; collect minimum necessary information.
Emergency contact	Name; relationship; phone	Text; mark optional unless clinical lead requires it.
Centre & allocation	Registered centre; current centre; assigned therapist; referred by	Dropdown/search; preserve centre-transfer history.
Lifestyle context	Occupation; work pattern; sitting/standing duration; physical activity	Structured choices plus notes.

Administration	Registration date; status; created by; last updated by	System-generated audit fields; status dropdown.
Duplicate check	Possible matching patient by phone/name/date of birth	Warn user; authorized staff confirm merge rather than silently merging.

Consent and privacy

Consent item	Required information	Behaviour
Assessment/documentation	Consent status, date, method, person obtaining consent	Required before assessment records are created, subject to approved clinical workflow.
Yoga practice	Consent status and date	Record separately from general privacy consent.
Physical adjustments	Consent for hands-on assistance where applicable	Must not be bundled with other permissions.
Reports/data sharing	Purpose, recipient/type of professional, status and date	Restrict sharing to authorized recipients.
Media	Photo/video permission and specific purpose	Optional and separate; refusal must not block therapy.
Communications	Permission for calls, SMS, email or WhatsApp reminders	Store channel-specific preference.
Consent record	Patient/guardian name, relationship if applicable, signature or documented method, version, staff member	Keep history of changes and withdrawal; do not overwrite old consent.
Privacy requests	Access/correction/withdrawal request, date, owner, action and closure date	Track resolution and audit it.

Medical case history

Subsection	Fields / prompts	Input design
Chief (main) complaint	Patient's own words; primary body region; side; main concern; first or recurring episode	Text plus dropdowns.
Onset & course	Onset date; duration; sudden/gradual; injury/activity associated; progression; previous episodes	Date, choices and narrative.
Pain/symptoms	Current/best/worst pain (0–10); quality; location/radiation; frequency; time pattern; stiffness; associated symptoms	Numeric score with scale label, multi-select and notes.
Aggravating/relieving factors	Movement, posture, work, rest, sleep or other factors	Multi-select plus free text.
Function	Walking, bending, lifting, sitting, standing, stairs, work, sleep or other limitations	Activity checklist; severity and notes.
Medical conditions	Known diagnoses, date/clinician if known, past injuries, surgery, hospitalization and relevant conditions	Repeatable entries with dates.
Medication/allergy	Patient-reported medicines, dose/frequency if known, prescriber if known, allergies and reaction	Repeatable entries; clearly label patient-reported.
Other providers	Treating doctor/specialist, other therapy, current restrictions	Text and optional contact details.
Lifestyle	Sleep hours/quality, exercise/walking, occupation, posture, work hours, relevant diet/hydration factors	Structured options plus notes; optional when not relevant.
Sensitive history	Pregnancy/postpartum, menstrual/menopause or other relevant history	Ask only when relevant, explain purpose and restrict access.
Patient goals	Desired activities, measurable goals, expectations, preferred frequency and	Text and goal records.

	barriers to attendance/home practice	

Safety screening and escalation

Screening prompt	Fields to capture	System response
New/severe/rapidly worsening symptoms	Yes/No/Unknown; description; screening date; reviewer	Flag for review; follow approved escalation procedure.
Recent significant trauma/fall	Answer, date and details	Prompt appropriate medical assessment where indicated.
New/progressive weakness or altered sensation	Answer, location, onset and notes	Flag for qualified clinical review.
New bladder/bowel control issue or saddle-area sensory change	Answer and time identified	High-priority warning; advise urgent medical evaluation according to approved protocol.
Fever/systemic symptoms/unexplained weight loss or concerning night pain	Answer and details	Flag for clinical review.
Chest pain, breathlessness, fainting or other acute symptoms	Answer and details	Emergency warning and urgent local care instructions where appropriate.
Decision and action	Pause/modify practice; referral advised; referred to; urgency; date/time; person responsible; outcome	No automatic diagnosis or clearance.

Assessment and tests

Subsection	Fields / prompts	Rules
Assessment metadata	Date/time; assessor; qualification/role; referral question; patient consent status	System timestamp plus staff selection.
General observations	Posture; gait; balance; mobility; relevant vitals if taken	Structured choices plus narrative; record units.
Movement/function	Region; movement; range; measurement and unit; side; pain/limitation; method	Repeatable measurement rows; allow unavailable/not tested with reason.
Strength/sensation	Finding, side, scale/method, notes	Only by staff qualified and authorized to perform the assessment.
SLR and special tests	Test name; indication; side; procedure/protocol; result; angle/unit if relevant; date; assessor; interpretation	Test library configurable; interpretation attributed to qualified professional.
Functional outcomes	Measure name/version; score; units; date; assessor	Use the same measure/version at follow-up where possible.
Clinical impression	Assessment summary; limitations; working impression; source/author	Separate documented diagnosis from therapist observations.
Safety decision	Proceed/modify/pause/refer; rationale; reviewer	Record decision, responsible professional and timestamp.

Region-specific form sections should appear conditionally:

- Cervical spine: neck movement, stiffness, pain pattern, shoulder symptoms and relevant neurological screening.
- Thoracic spine: upper-back mobility, posture, pain and movement limitations.
- Lumbar spine: back movement, pain distribution, lower-limb symptoms and relevant neurological screening.
- Pelvis/hip: hip movement, gait, pelvic mobility and relevant functional tests.
- Knee: pain, swelling, range of motion, alignment, gait and sit-to-stand function.
- Shoulder: movement, functional reach, pain and strength where appropriate.
- Ankle/foot: mobility, balance, gait and weight-bearing tolerance.
- General wellness: flexibility, balance, endurance and functional movement when relevant.

Investigations and attachments

Field	Requirement
Investigation type	X-ray, MRI, CT, ultrasound, laboratory test or other.
Body region/side	Structured field; allow multiple regions if applicable.

Investigation date	Date shown on the investigation/report.
Provider	Hospital/diagnostic centre and ordering/referring professional if known.
Report details	Report findings and impression copied accurately from the report; do not have the app invent interpretations.
Clinical review	Reviewer name/qualification, review date and attributed notes.
File	Secure file upload or protected document reference; permitted formats and file-size limits configured.
Comparison	Link to earlier investigation and note what is being compared.
Follow-up	Follow-up investigation requested, reason, due date and status.

Therapy plan

Subsection	Fields / prompts
Plan metadata	Plan ID; patient ID; date; version; prepared by/qualification; clinical reviewer if required; linked assessment.
Goals	Short-term and long-term goals; baseline; target; target review date; status.
Schedule	Sessions per week; proposed total/duration; session length; review date.
Practice item	Technique/asana name; Sanskrit/common name; category; body region; purpose; library reference.
Props/setup	Belt, brick, rope, loop band, tennis ball, chair, bolster, wall or other; starting position; setup; support/assistance.
Dose	Repetitions; hold duration; sets; rest interval; frequency; side.
Instructions	Breathing/teaching cues; approved modifications; home-practice instructions.
Safety	Precautions; contraindications; stop criteria; referral/co-management plan.
Approval/versioning	Patient agreement where applicable; author/reviewer; approved date; version; reason for change; previous version retained.

4Progress review

Field	Requirement
Review metadata	Review date; reviewer; sessions planned/attended/missed; home-practice adherence.
Comparison	Baseline and current pain score; range/function measurements; same outcome measure and score where available.
Goal status	Each goal marked achieved/partially achieved/not achieved/not assessed, with notes.
Patient feedback	Reported improvement/worsening, daily-function changes, concerns and satisfaction if collected.
Plan decision	Continue/modify/pause/refer; reason; new plan version; reviewer and date.
Next action	Next review date; pending medical review; follow-up owner.

Discharge and follow-up

Subsection	Fields
Closure	Discharge/transfer date; status; reason; total planned/completed sessions; closing staff member.
Outcome	Original goals; final assessment; final pain/function measures; goals achieved/partially achieved/not achieved.
Instructions	Home programme; precautions; warning signs explained; patient understanding confirmed.
Summary	Discharge summary generated from saved records; report provided and date.

Follow-up	Due date; contact method; contact outcome; symptoms reported; home adherence; next action.
Referral	Recommended provider/service, reason, date, urgency and whether follow-up was completed.
Case closure	Outstanding items; closure approval if required; record remains retrievable under retention policy.